

ANKLE CONDITIONS
DISABILITY BENEFITS QUESTIONNAIRE

Name of Patient/Veteran

Patient/Veteran's Social Security Number

Date of examination:

IMPORTANT - THE DEPARTMENT OF VETERANS AFFAIRS (VA) **WILL NOT PAY OR REIMBURSE** ANY EXPENSES OR COST INCURRED IN THE PROCESS OF COMPLETING AND/OR SUBMITTING THIS FORM.

Note - The Veteran is applying to the U.S. Department of Veterans Affairs (VA) for disability benefits. VA will consider the information you provide on this questionnaire as part of their evaluation in processing the Veteran's claim. VA may obtain additional medical information, including an examination, if necessary, to complete VA's review of the Veteran's application. VA reserves the right to confirm the authenticity of ALL completed questionnaires. **It is intended that this questionnaire will be completed by the Veteran's healthcare provider.**

Are you completing this Disability Benefits Questionnaire at the request of:

☐ Veteran/Claimant☐ Third party (please list name(s) of organization(s) or individual(s))☐ Other: please describe

Are you a VA Healthcare provider?

☐ Yes☐ No

Is the Veteran regularly seen as a patient in your clinic?

☐ Yes☐ No

Was the Veteran examined in person?

☐ Yes☐ No

If no, how was the examination conducted?

EVIDENCE REVIEW

Evidence reviewed:

☐ No records were reviewed☐ Records reviewed

Please identify the evidence reviewed (e.g. service treatment records, VA treatment records, private treatment records) and the date range.

SECTION 1 - DIAGNOSIS

Note: These are condition(s) for which an evaluation has been requested on the exam request form (Internal VA) or for which the Veteran has requested medical evidence be provided for submission to VA.

1A. List the claimed condition(s) that pertain to this questionnaire:

Note: These are the diagnoses determined during this current evaluation of the claimed condition(s) listed above. If there is no diagnosis, if the diagnosis is different from a previous diagnosis for this condition, or if there is a diagnosis of a complication due to the claimed condition(s), explain your findings and reasons in the Remarks section. Date of diagnosis can be the date of the evaluation if the clinician is making the initial diagnosis or an approximate date determined through record review or reported history.

1B. Select diagnoses associated with the claimed condition(s) (check all that apply):

☐ The Veteran does not have a current diagnosis associated with any claimed condition(s) listed above. (Explain your findings and reasons in the Remarks section)

	Side affected:			ICD code:	Date of diagnosis:		
	Right	Left	Both		Right:	Left:	
☐ Lateral collateral ligament sprain (chronic/recurrent)	☐	☐	☐	_____	_____	_____	_____
☐ Deltoid ligament sprain (chronic/recurrent)	☐	☐	☐	_____	_____	_____	_____
☐ Osteochondritis dissecans to include osteochondral fracture	☐	☐	☐	_____	_____	_____	_____
☐ Impingement (anterior/posterior (or trigonum syndrome)/anterolateral)	☐	☐	☐	_____	_____	_____	_____
☐ Tendonitis (Achilles/peroneal/posterior tibial)	☐	☐	☐	_____	_____	_____	_____
☐ Retrocalcaneal bursitis	☐	☐	☐	_____	_____	_____	_____
☐ Achilles' tendon rupture	☐	☐	☐	_____	_____	_____	_____
☐ Avascular necrosis, talus	☐	☐	☐	_____	_____	_____	_____
☐ Ankle joint replacement	☐	☐	☐	_____	_____	_____	_____
☐ Ankylosis of ankle, subtalar or tarsal joint	☐	☐	☐	_____	_____	_____	_____
☐ Shin splints/medial tibial stress syndrome - MTSS (including post-surgery or treatment)	☐	☐	☐	_____	_____	_____	_____

Note: If shin splints is diagnosed with compartment syndrome complete the Muscles questionnaire in lieu of this questionnaire.

☐ Degenerative arthritis, other than post-traumatic	☐	☐	☐	_____	Right:	Left:	_____
☐ Arthritis, gonorrheal	☐	☐	☐	_____	Right:	Left:	_____
☐ Arthritis, pneumococcic	☐	☐	☐	_____	Right:	Left:	_____
☐ Arthritis, streptococcic	☐	☐	☐	_____	Right:	Left:	_____
☐ Arthritis, syphilitic	☐	☐	☐	_____	Right:	Left:	_____
☐ Arthritis, rheumatoid (multi-joints)	☐	☐	☐	_____	Right:	Left:	_____
☐ Arthritis, post-traumatic	☐	☐	☐	_____	Right:	Left:	_____
☐ Arthritis, typhoid	☐	☐	☐	_____	Right:	Left:	_____
☐ Other specified forms of arthropathy (excluding gout):	_____			_____	Right:	Left:	_____
	☐	☐	☐	_____	Right:	Left:	_____
☐ Osteoporosis, residuals of	☐	☐	☐	_____	Right:	Left:	_____
☐ Osteomalacia, residuals of	☐	☐	☐	_____	Right:	Left:	_____
☐ Bones, neoplasm, benign	☐	☐	☐	_____	Right:	Left:	_____
☐ Bones, neoplasm, malignant, primary or secondary	☐	☐	☐	_____	Right:	Left:	_____
☐ Osteitis deformans	☐	☐	☐	_____	Right:	Left:	_____
☐ Gout	☐	☐	☐	_____	Right:	Left:	_____
☐ Bursitis	☐	☐	☐	_____	Right:	Left:	_____

☐ Myositis	☐ Right	☐ Left	☐ Both		Right:		Left:	
☐ Heterotopic ossification	☐ Right	☐ Left	☐ Both		Right:		Left:	
☐ Tendinopathy (select one if known)	☐ Right	☐ Left	☐ Both		Right:		Left:	
☐ Tendinitis	☐ Right	☐ Left	☐ Both		Right:		Left:	
☐ Tendinosis	☐ Right	☐ Left	☐ Both		Right:		Left:	
☐ Tenosynovitis	☐ Right	☐ Left	☐ Both		Right:		Left:	
☐ Other (specify):								
Other diagnosis #1: _____								
	☐ Right	☐ Left	☐ Both		Right:		Left:	
Other diagnosis #2: _____								
	☐ Right	☐ Left	☐ Both		Right:		Left:	
Other diagnosis #3: _____								
	☐ Right	☐ Left	☐ Both		Right:		Left:	

1C. If there are additional diagnoses that pertain to ankle conditions, list using above format:

SECTION 2 - MEDICAL HISTORY

2A. Describe the history, including onset and course, of the Veteran's ankle condition(s). Brief summary:

2B. Does the Veteran report flare-ups of the ankle?

☐ Yes ☐ No

If yes, document the Veteran's description of the flare-ups he/she experiences, including the frequency, duration, characteristics, precipitating and alleviating factors, severity, and/or extent of functional impairment he/she experiences during a flare-up of symptoms:

2C. Does the Veteran report having any functional loss or functional impairment of the joint or extremity being evaluated on this questionnaire, including but not limited to after repeated use over time?

☐ Yes ☐ No

If yes, document the Veteran's description of functional loss or functional impairment in his/her own words:

2D. Does the Veteran report or have a history of instability of the ankle?

☐ Yes ☐ No

If yes, document the Veteran's description of instability in his/her own words:

SECTION 3 - RANGE OF MOTION (ROM) AND FUNCTIONAL LIMITATION

There are several separate parameters requested for describing function of a joint. The question "Does this ROM contribute to a functional loss?" asks if there is a functional loss that can be ascribed to any documented loss of range of motion; and, unlike later questions, does not take into account the numerous other factors to be considered. Subsequent questions take into account additional factors such as pain, fatigue, weakness, lack of endurance, or incoordination. If there is pain noted on examination, it is important to understand whether or not that pain itself contributes to functional loss. Ideally, a claimant would be seen immediately after repetitive use over time or during a flare-up; however, this is not always feasible.

Information regarding joint function on repetitive use is broken up into two subsets. The first subset is based on observed repetitive use, and the second is based on functional loss associated with repeated use over time. The observed repetitive use section initially asks for objective findings after three or more repetitions of range of motion testing. The second subset provides a more global picture of functional loss associated with repetitive use over time. The latter takes into account medical probability of additional functional loss as a global view. This takes into account not only the objective findings noted on the examination, but also the subjective history provided by the claimant, as well as review of the available medical evidence.

Optimally, a description of any additional loss of function should be provided - such as what the degrees of range of motion would be opined to look like after repetitive use over time. However, when this is not feasible, an "as clear as possible" description of that loss should be provided. This same information (minus the three repetitions) is asked to be provided with regards to flare-ups.

RIGHT ANKLE

LEFT ANKLE

3A. Initial ROM measurements

☐ All Normal ☐ Abnormal or outside of normal range
☐ Unable to test ☐ Not indicated

If "Unable to test" or "Not indicated", please explain:

If ROM is outside of "normal" range, but is normal for the Veteran (for reasons other than an ankle condition, such as age, body habitus, neurologic disease), please describe:

3A. Initial ROM measurements

☐ All Normal ☐ Abnormal or outside of normal range
☐ Unable to test ☐ Not indicated

If "Unable to test" or "Not indicated", please explain:

If ROM is outside of "normal" range, but is normal for the Veteran (for reasons other than an ankle condition, such as age, body habitus, neurologic disease), please describe:

If abnormal, does the range of motion itself contribute to a functional loss?

☐ Yes ☐ No

If yes, please explain:

If abnormal, does the range of motion itself contribute to a functional loss?

☐ Yes ☐ No

If yes, please explain:

Note: For any joint condition, examiners should address pain on both passive and active motion, and on both weight-bearing and nonweight-bearing. Examiners should also test the contralateral joint (unless medically contraindicated). If testing cannot be performed or is medically contraindicated (such as it may cause the Veteran severe pain or the risk of further injury), an explanation must be given below. Please note any characteristics of pain observed on examination (such as facial expression or wincing on pressure or manipulation).

Can testing be performed?

☐ Yes ☐ No

If no, provide an explanation:

If this is the unclaimed joint, is it: ☐ Damaged ☐ Undamaged

If undamaged, range of motion testing must be conducted.

Active Range of Motion (ROM) - Perform active range of motion and provide the ROM values.

Plantar flexion endpoint (45 degrees): _____ degrees

Dorsiflexion endpoint (20 degrees): _____ degrees

If noted on examination, which ROM exhibited pain (select all that apply):

☐ Plantar flexion ☐ Dorsiflexion

If any limitation of motion is specifically attributable to pain, weakness, fatigability, incoordination, or other; please note the degree(s) in which limitation of motion is specifically attributable to the factors identified and describe.

_____ Plantar flexion degree endpoint (if different than above)

_____ Dorsiflexion degree endpoint (if different than above)

Passive Range of Motion - Perform passive range of motion and provide the ROM values.

Plantar flexion endpoint (45 degrees): _____ degrees ☐ Same as active ROM

Dorsiflexion endpoint (20 degrees): _____ degrees ☐ Same as active ROM

If noted on examination, which passive ROM exhibited pain (select all that apply):

☐ Plantar flexion ☐ Dorsiflexion

Can testing be performed?

☐ Yes ☐ No

If no, provide an explanation:

If this is the unclaimed joint, is it: ☐ Damaged ☐ Undamaged

If undamaged, range of motion testing must be conducted.

Active Range of Motion (ROM) - Perform active range of motion and provide the ROM values.

Plantar flexion endpoint (45 degrees): _____ degrees

Dorsiflexion endpoint (20 degrees): _____ degrees

If noted on examination, which ROM exhibited pain (select all that apply):

☐ Plantar flexion ☐ Dorsiflexion

If any limitation of motion is specifically attributable to pain, weakness, fatigability, incoordination, or other; please note the degree(s) in which limitation of motion is specifically attributable to the factors identified and describe.

_____ Plantar flexion degree endpoint (if different than above)

_____ Dorsiflexion degree endpoint (if different than above)

Passive Range of Motion - Perform passive range of motion and provide the ROM values.

Plantar flexion endpoint (45 degrees): _____ degrees ☐ Same as active ROM

Dorsiflexion endpoint (20 degrees): _____ degrees ☐ Same as active ROM

If noted on examination, which passive ROM exhibited pain (select all that apply):

☐ Plantar flexion ☐ Dorsiflexion

If any limitation of motion is specifically attributable to pain, weakness, fatigability, incoordination, or other; please note the degree(s) in which limitation of motion is specifically attributable to the factors identified and describe.

_____ Plantar flexion degree endpoint (if different than above)

_____ Dorsiflexion degree endpoint (if different than above)

Is there evidence of pain? ☐ Yes ☐ No

If yes, check all that apply.

- ☐ Weight-bearing ☐ Nonweight-bearing
- ☐ Active motion ☐ Passive motion ☐ On rest/non-movement
- ☐ Causes functional loss (if checked describe in the comments box below)
- ☐ Does not result in/cause functional loss

Comments:

Is there objective evidence of crepitus? ☐ Yes ☐ No

Is there objective evidence of localized tenderness or pain on palpation of the joint or associated soft tissue?

☐ Yes ☐ No

If yes, please explain. Include location, severity, and relationship to condition(s).

3B. Observed repetitive use ROM

Is the Veteran able to perform repetitive-use testing with at least three repetitions?

☐ Yes ☐ No

If no, please explain:

If any limitation of motion is specifically attributable to pain, weakness, fatigability, incoordination, or other; please note the degree(s) in which limitation of motion is specifically attributable to the factors identified and describe.

_____ Plantar flexion degree endpoint (if different than above)

_____ Dorsiflexion degree endpoint (if different than above)

Is there evidence of pain? ☐ Yes ☐ No

If yes, check all that apply.

- ☐ Weight-bearing ☐ Nonweight-bearing
- ☐ Active motion ☐ Passive motion ☐ On rest/non-movement
- ☐ Causes functional loss (if checked describe in the comments box below)
- ☐ Does not result in/cause functional loss

Comments:

Is there objective evidence of crepitus? ☐ Yes ☐ No

Is there objective evidence of localized tenderness or pain on palpation of the joint or associated soft tissue?

☐ Yes ☐ No

If yes, please explain. Include location, severity, and relationship to condition(s).

3B. Observed repetitive use ROM

Is the Veteran able to perform repetitive-use testing with at least three repetitions?

☐ Yes ☐ No

If no, please explain:

Is there additional loss of function or range of motion after three repetitions? ☐ Yes ☐ No If yes, please respond to the following after the completion of the three repetitions: Plantar flexion endpoint (45 degrees): _____ degrees Dorsiflexion endpoint (20 degrees): _____ degrees Select factors that cause this functional loss. Check all that apply. ☐ Pain ☐ Fatigability ☐ Weakness ☐ Lack of endurance ☐ Incoordination ☐ Other: _____ ☐ N/A	Is there additional loss of function or range of motion after three repetitions? ☐ Yes ☐ No If yes, please respond to the following after the completion of the three repetitions: Plantar flexion endpoint (45 degrees): _____ degrees Dorsiflexion endpoint (20 degrees): _____ degrees Select factors that cause this functional loss. Check all that apply. ☐ Pain ☐ Fatigability ☐ Weakness ☐ Lack of endurance ☐ Incoordination ☐ Other: _____ ☐ N/A
Note: When pain is associated with movement, the examiner must give a statement on whether pain could significantly limit functional ability during flare-ups and/or after repeated use over time in terms of additional loss of range of motion. In the exam report, the examiner is requested to provide an estimate of decreased range of motion (in degrees) that reflect frequency, duration, and during flare-ups - even if not directly observed during a flare-up and/or after repeated use over time.	
3C. Repeated use over time Is the Veteran being examined immediately after repeated use over time? ☐ Yes ☐ No Does procured evidence (statements from the Veteran) suggest pain, fatigability, weakness, lack of endurance, or incoordination which significantly limits functional ability with repeated use over time? ☐ Yes ☐ No Select factors that cause this functional loss. Check all that apply. ☐ Pain ☐ Fatigability ☐ Weakness ☐ Lack of endurance ☐ Incoordination ☐ Other: _____ ☐ N/A Estimate range of motion in degrees for this joint immediately after repeated use over time based on information procured from relevant sources including the lay statements of the Veteran. Plantar flexion endpoint (45 degrees): _____ degrees Dorsiflexion endpoint (20 degrees): _____ degrees The examiner should provide the estimated range of motion based on a review of all procurable information - to include the Veteran's statement on examination, case-specific evidence (to include medical treatment records when applicable and lay evidence), and the examiner's medical expertise. If, after evaluation of the procurable and assembled data, the examiner determines that it is not feasible to provide this estimate, the examiner should explain why an estimate cannot be provided. The explanation should not be based on an examiner's shortcomings or a general aversion to offering an estimate on issues not directly observed. Please cite and discuss evidence. (Must be specific to the case and based on all procurable evidence.)	3C. Repeated use over time Is the Veteran being examined immediately after repeated use over time? ☐ Yes ☐ No Does procured evidence (statements from the Veteran) suggest pain, fatigability, weakness, lack of endurance, or incoordination which significantly limits functional ability with repeated use over time? ☐ Yes ☐ No Select factors that cause this functional loss. Check all that apply. ☐ Pain ☐ Fatigability ☐ Weakness ☐ Lack of endurance ☐ Incoordination ☐ Other: _____ ☐ N/A Estimate range of motion in degrees for this joint immediately after repeated use over time based on information procured from relevant sources including the lay statements of the Veteran. Plantar flexion endpoint (45 degrees): _____ degrees Dorsiflexion endpoint (20 degrees): _____ degrees The examiner should provide the estimated range of motion based on a review of all procurable information - to include the Veteran's statement on examination, case-specific evidence (to include medical treatment records when applicable and lay evidence), and the examiner's medical expertise. If, after evaluation of the procurable and assembled data, the examiner determines that it is not feasible to provide this estimate, the examiner should explain why an estimate cannot be provided. The explanation should not be based on an examiner's shortcomings or a general aversion to offering an estimate on issues not directly observed. Please cite and discuss evidence. (Must be specific to the case and based on all procurable evidence.)
3D. Flare-ups Is the examination being conducted during a flare-up? ☐ Yes ☐ No Does procured evidence (statements from the Veteran) suggest pain, fatigability, weakness, lack of endurance, or incoordination which significantly limits functional ability with flare-ups? ☐ Yes ☐ No Select factors that cause this functional loss. Check all that apply. ☐ Pain ☐ Fatigability ☐ Weakness ☐ Lack of endurance ☐ Incoordination ☐ Other: _____ ☐ N/A	3D. Flare-ups Is the examination being conducted during a flare-up? ☐ Yes ☐ No Does procured evidence (statements from the Veteran) suggest pain, fatigability, weakness, lack of endurance, or incoordination which significantly limits functional ability with flare-ups? ☐ Yes ☐ No Select factors that cause this functional loss. Check all that apply. ☐ Pain ☐ Fatigability ☐ Weakness ☐ Lack of endurance ☐ Incoordination ☐ Other: _____ ☐ N/A

Estimate range of motion in degrees for this joint during flare-ups based on information procured from relevant sources including the lay statements of the Veteran.

Plantar flexion endpoint (45 degrees): _____ degrees

Dorsiflexion endpoint (20 degrees): _____ degrees

The examiner should provide the estimated range of motion based on a review of all procurable information - to include the Veteran's statement on examination, case-specific evidence (to include medical treatment records when applicable and lay evidence), and the examiner's medical expertise. If, after evaluation of the procurable and assembled data, the examiner determines that it is not feasible to provide this estimate, the examiner should explain why an estimate cannot be provided. The explanation should not be based on an examiner's shortcomings or a general aversion to offering an estimate on issues not directly observed.

Please cite and discuss evidence. (Must be specific to the case and based on all procurable evidence.)

Estimate range of motion in degrees for this joint during flare-ups based on information procured from relevant sources including the lay statements of the Veteran.

Plantar flexion endpoint (45 degrees): _____ degrees

Dorsiflexion endpoint (20 degrees): _____ degrees

The examiner should provide the estimated range of motion based on a review of all procurable information - to include the Veteran's statement on examination, case-specific evidence (to include medical treatment records when applicable and lay evidence), and the examiner's medical expertise. If, after evaluation of the procurable and assembled data, the examiner determines that it is not feasible to provide this estimate, the examiner should explain why an estimate cannot be provided. The explanation should not be based on an examiner's shortcomings or a general aversion to offering an estimate on issues not directly observed.

Please cite and discuss evidence. (Must be specific to the case and based on all procurable evidence.)

3E. Additional factors contributing to disability

In addition to those addressed above, are there additional contributing factors of disability? Please select all that apply and describe:

- | | |
|---|--|
| ☐ None | ☐ Interference with sitting |
| ☐ Interference with standing | ☐ Swelling |
| ☐ Disturbance of locomotion | ☐ Deformity |
| ☐ Less movement than normal | ☐ More movement than normal |
| ☐ Weakened movement | ☐ Atrophy of disuse |
| ☐ Instability of station | |
| ☐ Other, describe: _____ | |

Please describe additional contributing factors of disability:

3E. Additional factors contributing to disability

In addition to those addressed above, are there additional contributing factors of disability? Please select all that apply and describe:

- | | |
|---|--|
| ☐ None | ☐ Interference with sitting |
| ☐ Interference with standing | ☐ Swelling |
| ☐ Disturbance of locomotion | ☐ Deformity |
| ☐ Less movement than normal | ☐ More movement than normal |
| ☐ Weakened movement | ☐ Atrophy of disuse |
| ☐ Instability of station | |
| ☐ Other, describe: _____ | |

Please describe additional contributing factors of disability:

SECTION 4 - MUSCLE ATROPHY

4A. Does the Veteran have muscle atrophy?

☐ Yes ☐ No

4B. If yes, is the muscle atrophy due to the claimed condition in the diagnosis section?

☐ Yes ☐ No

If no, provide rationale:

4A. Does the Veteran have muscle atrophy?

☐ Yes ☐ No

4B. If yes, is the muscle atrophy due to the claimed condition in the diagnosis section?

☐ Yes ☐ No

If no, provide rationale:

4C. For any muscle atrophy due to a diagnosis listed in Section I, indicate specific location of atrophy, providing measurements in centimeters of normal side and corresponding atrophied side, measured at maximum muscle bulk.

☐ Right lower extremity (specify location of measurement such as "1cm above or below ankle"):

Circumference of more normal side: _____ cm

Circumference of atrophied side: _____ cm

4C. For any muscle atrophy due to a diagnosis listed in Section I, indicate specific location of atrophy, providing measurements in centimeters of normal side and corresponding atrophied side, measured at maximum muscle bulk.

☐ Left lower extremity (specify location of measurement such as "1cm above or below ankle"):

Circumference of more normal side: _____ cm

Circumference of atrophied side: _____ cm

SECTION 5 - ANKYLOSIS

Note: Ankylosis is the immobilization of a joint due to disease, injury or surgical procedure.

5A. Is there ankylosis of the ankle? ☐ Yes ☐ No

If yes, indicate the severity of ankle ankylosis:

- ☐ In plantar flexion, less than 30 degrees
- ☐ In plantar flexion, between 30 degrees and 40 degrees
- ☐ In plantar flexion at more than 40 degrees
- ☐ In dorsiflexion, between 0 degrees and 10 degree
- ☐ In dorsiflexion at more than 10 degrees
- ☐ With an abduction deformity
- ☐ With an adduction deformity
- ☐ With an inversion deformity
- ☐ With an eversion deformity

5B. Indicate angle of ankle ankylosis in ☐ N/A no ankle ankylosis of joint degrees.

Plantar flexion: _____

Dorsiflexion: _____

5A. Is there ankylosis of the ankle? ☐ Yes ☐ No

If yes, indicate the severity of ankle ankylosis:

- ☐ In plantar flexion, less than 30 degrees
- ☐ In plantar flexion, between 30 degrees and 40 degrees
- ☐ In plantar flexion at more than 40 degrees
- ☐ In dorsiflexion, between 0 degrees and 10 degree
- ☐ In dorsiflexion at more than 10 degrees
- ☐ With an abduction deformity
- ☐ With an adduction deformity
- ☐ With an inversion deformity
- ☐ With an eversion deformity

5B. Indicate angle of ankle ankylosis in ☐ N/A no ankle ankylosis of joint degrees.

Plantar flexion: _____

Dorsiflexion: _____

5C. Is there ankylosis of the subastragalar or tarsal joint?

- ☐ Yes ☐ No If yes, indicate severity:
- ☐ In good weight-bearing position
 - ☐ In poor weight-bearing position

5C. Is there ankylosis of the subastragalar or tarsal joint?

- ☐ Yes ☐ No If yes, indicate severity:
- ☐ In good weight-bearing position
 - ☐ In poor weight-bearing position

SECTION 6 - JOINT STABILITY

6A. Complete the following:

Anterior Drawer Test: Is there absence of firm end point with asymmetric or excessive motion?

☐ Yes ☐ No ☐ Unable to test

6A. Complete the following:

Anterior Drawer Test: Is there absence of firm end point with asymmetric or excessive motion?

☐ Yes ☐ No ☐ Unable to test

Talar Tilt Test: Is there asymmetric or excessive motion? ☐ Yes ☐ No ☐ Unable to test If unable to test, please explain why:	Talar Tilt Test: Is there asymmetric or excessive motion? ☐ Yes ☐ No ☐ Unable to test If unable to test, please explain why:
6B. If unable to test, is ankle instability suspected? ☐ Yes ☐ No If yes, please describe:	6B. If unable to test, is ankle instability suspected? ☐ Yes ☐ No If yes, please describe:
SECTION 7 - ADDITIONAL COMMENTS	
7A. Does the Veteran now have or has he or she ever had shin splints (medial tibial stress syndrome), stress fractures, Achilles tendonitis, Achilles tendon rupture, malunion of calcaneus (os calcis) or talus (astragalus), or has the Veteran had a talectomy (astragalectomy)? ☐ Yes ☐ No If yes, indicate condition and complete the appropriate sections below: ☐ Stress fracture of the lower leg (If this affects ROM of the knee, please complete the appropriate musculoskeletal questionnaire and ROM section) Describe current symptoms: ☐ Achilles tendonitis or Achilles tendon rupture Describe current symptoms: ☐ Malunion of calcaneus (os calcis) or talus (astragalus) Indicate severity: ☐ Moderate deformity ☐ Marked deformity ☐ "Shin Splints" (medial tibial stress syndrome - MTSS) Indicate length of treatment: ☐ no treatment received ☐ treatment for less than 12 consecutive months ☐ requiring treatment for 12 consecutive months or more	7A. Does the Veteran now have or has he or she ever had shin splints (medial tibial stress syndrome), stress fractures, Achilles tendonitis, Achilles tendon rupture, malunion of calcaneus (os calcis) or talus (astragalus), or has the Veteran had a talectomy (astragalectomy)? ☐ Yes ☐ No If yes, indicate condition and complete the appropriate sections below: ☐ Stress fracture of the lower leg (If this affects ROM of the knee, please complete the appropriate musculoskeletal questionnaire and ROM section) Describe current symptoms: ☐ Achilles tendonitis or Achilles tendon rupture Describe current symptoms: ☐ Malunion of calcaneus (os calcis) or talus (astragalus) Indicate severity: ☐ Moderate deformity ☐ Marked deformity ☐ "Shin Splints" (medial tibial stress syndrome - MTSS) Indicate length of treatment: ☐ no treatment received ☐ treatment for less than 12 consecutive months ☐ requiring treatment for 12 consecutive months or more

If Veteran underwent treatment, indicate response to treatment:

- ☐ responsive to surgery and/or treatment
- ☐ unresponsive to either shoe orthotics or other conservative treatment
- ☐ unresponsive to surgery and either shoe orthotics or other conservative treatment

Does this condition affect ROM of knee?

- ☐ Yes (If yes, complete the Knee and Lower Leg Conditions questionnaire)
- ☐ No

Describe current symptoms:

☐ Talcotomy

Describe current symptoms:

If Veteran underwent treatment, indicate response to treatment:

- ☐ responsive to surgery and/or treatment
- ☐ unresponsive to either shoe orthotics or other conservative treatment
- ☐ unresponsive to surgery and either shoe orthotics or other conservative treatment

Does this condition affect ROM of knee?

- ☐ Yes (If yes, complete the Knee and Lower Leg Conditions questionnaire)
- ☐ No

Describe current symptoms:

☐ Talcotomy

Describe current symptoms:

SECTION 8 - SURGICAL PROCEDURES

8A. Indicate any surgical procedures that the Veteran has had performed and provide the additional information as requested (check all that apply):

- ☐ No surgery
- ☐ Total ankle joint replacement

Date of surgery: _____

Residuals:

- ☐ None
- ☐ Intermediate degrees of residual weakness, pain or limitation of motion
- ☐ Chronic residuals consisting of severe painful motion or weakness
- ☐ Other, describe:

☐ Arthroscopic or other ankle surgery

Type of surgery: _____

Date of surgery: _____

8A. Indicate any surgical procedures that the Veteran has had performed and provide the additional information as requested (check all that apply):

- ☐ No surgery
- ☐ Total ankle joint replacement

Date of surgery: _____

Residuals:

- ☐ None
- ☐ Intermediate degrees of residual weakness, pain or limitation of motion
- ☐ Chronic residuals consisting of severe painful motion or weakness
- ☐ Other, describe:

☐ Arthroscopic or other ankle surgery

Type of surgery: _____

Date of surgery: _____

☐ Residuals of arthroscopic or other ankle surgery

Describe residuals:

☐ Residuals of arthroscopic or other ankle surgery

Describe residuals:

SECTION 9 - OTHER PERTINENT PHYSICAL FINDINGS, COMPLICATIONS, CONDITIONS, SIGNS, SYMPTOMS, AND SCARS

9A. Does the Veteran have any other pertinent physical findings, complications, conditions, signs or symptoms related to any conditions listed in the diagnosis section above?

☐ Yes ☐ No

If yes, describe (brief summary):

9B. Does the Veteran have any scars or other disfigurement (of the skin) related to any conditions or to the treatment of any conditions listed in the diagnosis section?

☐ Yes ☐ No If yes, also complete the appropriate dermatological questionnaire.

SECTION 10 - ASSISTIVE DEVICES

10A. Does the Veteran use any assistive devices as a normal mode of locomotion, although occasional locomotion by other methods may be possible?

☐ Yes ☐ No

If yes, identify the assistive devices used (check all that apply and indicate frequency):

☐ Wheelchair	Frequency of use:	☐ Occasional	☐ Regular	☐ Constant
☐ Brace(s)	Frequency of use:	☐ Occasional	☐ Regular	☐ Constant
☐ Crutch(es)	Frequency of use:	☐ Occasional	☐ Regular	☐ Constant
☐ Cane(s)	Frequency of use:	☐ Occasional	☐ Regular	☐ Constant
☐ Walker	Frequency of use:	☐ Occasional	☐ Regular	☐ Constant
☐ Other: _____	Frequency of use:	☐ Occasional	☐ Regular	☐ Constant

10B. If the Veteran uses any assistive devices, specify the condition, indicate the side, and identify the assistive device used for each condition.

SECTION 11 - REMAINING EFFECTIVE FUNCTION OF THE EXTREMITIES

Note: The intention of this section is to permit the examiner to quantify the level of remaining function; it is not intended to inquire whether the Veteran should undergo an amputation with fitting of a prosthesis. For example, if the functions of grasping (hand) or propulsion (foot) are as limited as if the Veteran had an amputation and prosthesis, the examiner should check "yes" and describe the diminished functioning. The question simply asks whether the functional loss is to the same degree as if there were an amputation of the affected limb.

11A. Due to the Veterans ankle condition(s), is there functional impairment of an extremity such that no effective function remains other than that which would be equally well served by an amputation with prosthesis (functions of the lower extremity include balance and propulsion, etc.)?

☐ Yes, functioning is so diminished that amputation with prosthesis would equally serve the Veteran.

☐ No

If yes, indicate extremities for which this applies:

☐ Right lower

☐ Left lower

11B. For each checked extremity, identify the condition causing loss of function, describe loss of effective function and provide specific examples (brief summary):

SECTION 12 - DIAGNOSTIC TESTING

Note: Testing listed below is not indicated for every condition. The diagnosis of degenerative arthritis (osteoarthritis) or post-traumatic arthritis must be confirmed by imaging studies. Once such arthritis has been documented, even if in the past, no further imaging studies are required by VA, even if arthritis has worsened.

12A. Have clinically relevant diagnostic imaging studies or other diagnostic procedures been performed or reviewed in conjunction with this examination? ☐ Yes ☐ No

12B. If yes, is degenerative or post-traumatic arthritis documented? ☐ Yes ☐ No

If yes, indicate side:

☐ Right

☐ Left

☐ Both

12C. If yes, provide type of test or procedure, date and results (brief summary):

12D. Are there any other clinically relevant diagnostic test findings or results related to the claimed condition(s) and/or diagnosis(es), that were reviewed in conjunction with this examination?

☐ Yes ☐ No

If yes, provide type of test or procedure, date and results (brief summary):

12E. If any test results are other than normal, indicate relationship of abnormal findings to diagnosed conditions:

SECTION 13 - FUNCTIONAL IMPACT

Note: Provide the impact of only the diagnosed condition(s), without consideration of the impact of other medical conditions or factors, such as age.

13A. Regardless of the Veteran's current employment status, do the conditions listed in the diagnosis section impact his/her ability to perform any type of occupational task (such as standing, walking, lifting, sitting, etc.)?

☐ Yes ☐ No

If yes, describe the functional impact of each condition, providing one or more examples:

SECTION 14 - REMARKS

14A. Remarks (if any - please identify the section to which the remark pertains when appropriate).

SECTION 15 - EXAMINER'S CERTIFICATION AND SIGNATURE

CERTIFICATION - To the best of my knowledge, the information contained herein is accurate, complete and current.

PENALTY: The law provides severe penalties which include fine or imprisonment, or both, for the willful submission of any statement or evidence of a material fact, knowing it to be false, or for the fraudulent acceptance of any payment to which you are not entitled.

15A. Examiner's signature:

15B. Examiner's printed name and title (e.g. MD, DO, DDS, DMD, Ph.D, Psy.D, NP, PA-C):

15C. Examiner's area of practice/specialty (e.g. Cardiology, Orthopedics, Psychology/Psychiatry, General Practice):

15D. Date signed:

15E. Examiner's phone/fax numbers:

15F. National Provider Identifier (NPI) number:

15G. Medical license number and state:

15H. Examiner's address: